جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : ايمان طه حسن على
المحافظة : القاهره **الحي / المدينة :** شبرا **العنوان :** --
رقم البطاقة : 25706250102164 **رقم التليفون :** 01128798888 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 25-06-1957 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام اشعة مقطعية مخ مع صبغة [10851] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ايمان طه حسن علي
National ID: 25706250102164

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of the brain.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





Dms

- مزيد من البحث و التحاليل
- ترومبتا

جوستاف روسي

٢٠٢٦٠٥٠١٠٤١٢٤

Patient's Name:	**EMAN TAHA HASAN ALI**
Date:	**2026-05-05**
Referred by:	**Prof.Dr. CT CAP+C**
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported right breast carcinoma , compared to last study dated 27/1/2026 .

(I) Chest:

- **Right deep seated breast mass inseparable from underlying pectoralis muscle measures 2.7 x 2.9 cm .**
- **Still noted right lower lobe posterior segment pleural based pulmonary nodules 12 x 12 mm .**
- **No sizable hilar or mediastinal lymph nodes .**
- **Clear both lung fields .no sizable pulmonary masses or consolidation patches**
- **No pleural or pericardial collections seen.**
- **No gross cardiac abnormality.**

(II) Abdomen and pelvis:

- **The liver is enlarged in size showing homogenous low CT density no focal lesion**

CT Scan
[DOCTYPE CT SCAN]

Name: ايمان طه حسن علي	
MR. : 4079	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

تشرف بأن توضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presented for initiation of faslodex and ibrance 125 mg; laboratory studies were within normal limits.

The patient is a 68-year-old female with breast cancer, ER 8/8, PR 4/8, HER2-positive (KI6730 %). She received neoadjuvant chemotherapy with AC followed by taxol, underwent breast conserving surgery (BCS) with T2N2 disease and 2/12 lymph nodes positive, and had a PORT in December 2020. She was lost to follow-up for 7 months. Multiple lung nodules were noted; ibrance and faslodex were started at the time of the CTCAP visit on 27 January 2026. A few bilateral scattered pulmonary nodules were seen, the largest measuring 12 × 12 mm in the right lower lobe, for which ibrance 125 mg and faslodex 500 mg were prescribed.

The patient returned on 3 May 2026 for continuation of faslodex and ibrance 125 mg; laboratory studies remained normal. A new CT chest was obtained and compared to the previous study.

On 31 May 2026, a CT chest was reviewed, showing stable disease. The patient continued ibrance 125 mg and faslodex. Treatment was pending approval, and pharmacy was notified that this cycle would be administered once approved.

تحريرا في ,

التاريخ :	19-09-2026	الوقت :	

الطبيب المعالج : Reham Ahmed Fathy Ibrahim

يعتمد ,

جوستاف روسي

Patient's Name:	EMAN TAHA HASAN ALI
Date:	2026-05-05
Referred by:	Prof.Dr. CT CAP+C
Hospital:	Goustave Roussey International Egypt

- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys (bilateral cortical cysts), aorta and IVC.
- Normal CT appearance of the spleen .
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- No worrisome lytic or sclerotic bony lesions
- No vertebral structural collapse .

OPINION:.

Overall stationary course since the last study

Dr.Reham Khedr, MD

CT Scan
[DOCTYPE:CT_SCAN]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : ايمان طه حسن علي
2-ID number: 25706250102164

Sex: Female

3- Medical number: 83149156
Old Med : 5331/2019

Patient Age : 66 Y

Family history		Phone	01128798887	BMI		REASON OF MDT	REMDT
marital status		NRS Score		GROUP	M	Date of MDT	2023-08-01
Co morbidity		Menopausal status					

Clinical data

Breast Imaging :	Mets Workup :

BX:	ER:	PR:	Her 2 Neu :	SISH :	Ki67 :
	8/8	4/8	0		30

T	N	M

Sonomammography

PATIENT HISTORY

RECEIVED NACTH AC THEN TAXOL-->BCS T2N2/12-->PORTH
12/2020-->FEMARA -->LOST FU
7/2023-->MULTIPLE LUNG NODULES

MDT OUTCOME & PLAN

for fasoldex+ibrance+new bone scan

SIGNATURE

MEDICAL ONCOLOGY	د/ شريف حسين	RADIOLOGY	د/ ايمان الدراغ
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[DOCTYPE MDT_FORM]

Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m6939

اسم المريض	: ايمان طه حسن	النوع	: Female
السن	: 62 Y	العنوان	: القاهرة
تاريخ اخذ العينة	:	تاريخ استلام العينة	: 02/11/2019 11:19
طالب التحليل	: صحة المراة	تاريخ التقرير	: 26/11/2019 11:14
عناية	: صحة المراة		

Primary Sample :

Paraffin Block

Test :

ER &PR & HER2 neu & Ki67 (ER&PR Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.
Sample recieved was treated by : -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.
Sample recived was treated by :- CONFIRM anti Ki67 (30-9) Rabbeit monoclonal primary antibody.

Result :

ER: strong nuclear staining of > 60 % of tumour cells (core biopsy)
PR: moderate nuclear staining of 1 % of tumour cells (core biopsy)
HER2/neu protein over expression : NO membrane staining of tumor cells(core biopsy) .
Ki67 : about > 30 % of tumour cells are positive for ki67 (core biopsy)

Referal Lab :

Comment :

Conclusioh :

ER: POSITIVE (SCORE 8 / 8)
PR: POSITIVE (SCORE 4 / 8)
HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE
#Ki67: THE PROLIFERATION INDEX OF KI67 IS ABOUT > 30 % OF THE TUMOUR CELLS

Pathologist: د/تريزا ماهر

Lab Director: د/ تريزا ماهر

19 El-sheikh Rehan Street. Abeideen. Cairo
Tel.(165)7948544 -7950096 Fax. (202)7948544
Mobile . 01144297292
website http://www.healthlabs.net
email:pathologychl@yahoo.com
facebook: قسم الباثولوجى - الإدارة المركزية للمعامل

This Test Is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها

Immunohistochemistry
[DOCTYPE HCE]

Ministry of health
Central health laboratories
Pathology department

Histopathology report

Report Number: 636

اسم المريض : ايمان طه حسن		النوع : Female	
السن : 62 Y		العنوان : القاهرة	
تاريخ اخذ العينة :		تاريخ استلام العينة : 18/11/2019 00:00	
طالب التحليل : صحة المرأة		تاريخ التقرير : 20/11/2019 11:13	
عناية : صحة المرأة			

Primary sample :
east core biopsy

comment :

Gross examination :
res of greyish white tissue (T.I)

Microscopic examination :
rocollagenous tissue infiltrated by malignant growth displaying the morphology of infiltrating duct carcinoma

Referal Lab :

Comment :

Diagnosis :
EAST CORES: THE PICTURE IS COMPATIBLE WITH INFILTRATING DUCT CARCINOMA G II

athologist: Dr\Jehan Aziz

Lab Director: د/ تريزا ماهر

sheikh Rehan Street, Abeldeen, Cairo
165)7948544 -7950096 Fax. (202)7948544
e. 01144297292
ite:http://www.healthlabs.net
:pathologych!@yahoo.com
ook: قسم الباثولوجي - الادارة المركزية للمعامل

This Test Is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها

Pathology Report
[[DOCTYPE PATHOLOGY_REPORT]]

امانة المراكز الطبية المتخصصة
مركز اورام دار السلام
اداره المعمل
قسم الباثولوجي

Pathology Report

Name: ايمان طه حسن Age: 62 years.

Clinical data: Right breast cancer, post chemotherapy.

Specimen type: Right upper, outer quadrentectomy & axillary clearance.

Date: 19/7/2020

Lab. Number: 1022 Hospital Number: 5331/2019 مبادرة

Gross Description:

A sector of mammary tissue; 12x6.5x4.5 cm, with partial skin covering; 11x4 cm, with attached axillary tissue; 8x6 cm, received enblock.

Serial cuts revealed an ill-defined clip-marked firm fibrotic greyish white mass; 3x2.5 cm, located; 1.4 cm deep to skin and located (2, 2, 4.5, 4 & 1.5) cm from (lateral, medial, superior, inferior & deep) margins respectively.

Dissection of axillary tissue revealed 12 nodules, up to; 3.5x2.5 cm.

Microscopic Examination:

Histopathological examination of section prepared revealed remnants of invasive tumor cells, in form of groups of anaplastic ductal epithelial cells, with areas of fat necrosis, fibrosis & inflammation.

Margins were free.

Lymph nodes showed deposits in 2 nodes (2/12).

Diagnosis:

Right breast upper, outer quadrentectomy & axillary clearance, post chemotherapy:

- **Remnants of invasive duct carcinoma GII (Partial pathological response to therapy).**
- **Margins were free.**
- **Lymph node status; 2/12.**
- **Stage ypT2 ypN1.**

Prof. Dr. Gamal Abdel-Aty
Dr. Rana Fathy

Prof. Dr. Essam Ayad

Pathology Report
WWW.DAR-PATHOLOGY-REPORTS

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Eman Taha Hassan Ali	Date : 04/07/2023
Exam : C.T CHEST	Age : 66 Year(s)
Medical ID: 5331/2019	Pat. ID: 39719

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS: -

Findings:

- Status post right BCS with no operative bed definite mass and overlying skin thickening.
- Right upper lung and middle lung lobes anteriorly subpleural fine reticulonodular densities likely post radiotherapy sequele.
- Bilateral pulmonary multiple scattered tiny nodules are seen, the largest is seen at the right lower lung lobe superior segment measuring about 1.1 cm.
- Bilateral diffuse predominantly upper lobar emphysematous chnages are seen.
- No hilar or mediastinal sizable lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Hiatus hernia.
- Bilateral elastofibrpma dorsi are seen.

II Abdomen & pelvis:

- The liver is enlarged showing diffuse hypodense CT texture. No focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Gall bladder bed shows surgical clips of previous cholecystectomy.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Right suprarenal soft tissue nodule is seen measuring about 17x16 mm.

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Eman Taha Hassan Ali	Date : 04/07/2023
Exam : C.T CHEST	Age : 66 Year(s)
Medical ID: 5331/2019	Pat. ID: 39719

- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Bilateral renal cortical cysts.
- Normal appearance of the pancreas, aorta, I.V.C and left suprarenal.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Skeletal examination revealed:
  - Dorsolumbar spondylotic changes.
  - No sizable lytic or sclerotic bone lesions.

Opinion:

- **Status post right BCS with no operative bed definite mass and overlying skin thickening.**
- **Bilateral pulmonary multiple scattered tiny nodules, likely metastatic.**
- **Right suprarenal soft tissue nodule, for dynamic contrast study.**
- **Fatty hepatomegaly.**
- **Cholecystectomy.**

Dr. Reham Anwar, FEBR, MD

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Exam Information:

Patient Name: *Eman Taha*	***Date of birth:*** *25/06/1957*
Arabic Name: ايمان طه	***Patient Gender:*** *F*
Patient RIS ID: *241799* ***Patient HIS ID:*** *119024730*	***Exam Date:*** *11/09/2023*
Written by: Dr. Hoda Fathy	**Approved by:** Dr. Hoda Fathy

^{99m}Tc MDP Bone Scan Report (11-9-2023)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP

Revealed:

- No current definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Exaggerated degenerative changes are noted over the upper cervical and lower dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No current definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged

Dr. Nada Fadl, MSc

Bone Scan
[DOCTYPE:BONE_SCAN]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Eman Taha Hassan Ali	Date : 04/07/2023
Exam : MAMMOGRAPHY BOTH	Age : 66 Year(s)
Medical ID: 5331/2019	Pat. ID: 39719

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had right CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- No suspicious mass lesions, micro calcifications or architectural distortion detected.
- right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Yours truly

Signature Dr. Walaa Abdelazez

04/07/2023 1:14 PM

Mammogram
[[DOCTYPE MAMMOGRAM]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Name : eman taha hasan	**Date : 27-01-2026 11:23**
Branch : Portable	**Medical No. : GRI-98962**
Ref.Physician Prof : cap +c	**Ins.Company :**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- Few bilateral scattered pulmonary nodules largest measuring 12 x 12 mm at the right lower lobe.
- Bilateral emphysematous lung changes in the form of multiple scattered areas of air trapping and dilated parenchymal air spaces.
- No pulmonary masses.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes. Few bilateral scattered renal cortical cysts.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ **Few bilateral scattered pulmonary nodules for PET-CT assessment.**

❖ **Hepatomegaly.**

Dr. Sherif Wagdy

Dr. Sherif Wagdy

CT Scan
[DOCTYPE:CT_SCAN]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Name : eman taha hasan	Date : 27-01-2026 11:23
Branch : Portable	Medical No. : GRI-98962
Ref.Physician Prof : cap tc	Ins.Company :

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- Few bilateral scattered pulmonary nodules largest measuring 12 x 12 mm at the right lower lobe.
- Bilateral emphysematous lung changes in the form of multiple scattered areas of air trapping and dilated parenchymal air spaces.
- No pulmonary masses.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes. Few bilateral scattered renal cortical cysts.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ **Few bilateral scattered pulmonary nodules for PET-CT assessment.**

❖ **Hepatomegaly.**

Dr. Sherif Wagdy

Dr. Sherif Wagdy

CT Scan
[DOCTYPE:CT_SCAN]

GUSTAVE ROUSSY
INTERNATIONAL

Name : Eman Taha Hassan Ali	**Date : 23-10-2025 11:15**
Branch : Portable	**Medical No. : GRI-39719**
Ref.Physician Prof :	**Ins.Company :**

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Compared to the last study dated 9-7-2025 ,the current study revealed:

(I) Chest:

- **Newly seen** bilateral centrilobular emphysematous chnages are noted with relative sparing of the lung bases.
- Right middle lobar subpleural reticulations of post-radiation therapy sequel.
- **Still noted** right middle lobar medial segmental small pleural based nodule is seen measuring 6.5x6 mm in diameter ? metastatic, for PET-CT correlation and follow up.
- The rest of both lung fields showing no other parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement in the current study .
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Right breast postoperative changes ,for sonomammographic correlation.

(II) Abdomen and pelvis:

CT Scan
(DOCTYPE:CT SCAN)

GUSTAVE ROUSSY
INTERNATIONAL

Name : Eman Taha Hassan Ali	**Date : 23-10-2025 11:15**
Branch : Portable	**Medical No. : GRI-39719**
Ref.Physician Prof :	**Ins.Company :**

Stationary course of the previously noted:

- The liver is mildly enlarged in size showing homogenous reduced CT density with no focal lesions.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Cholecystectomy clips are noted.
- Normal CT appearance of the spleen, pancreas, left suprarenal gland, both kidneys, apart from left renal cortical simple cyst , measuring 4x3.9 cm in diameter, aorta and IVC.
- Bulky right suprarenal gland with a nodule meausirng1 5x11 mm .
- Few small right renal cortical cysts.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.

OPINION:

- **Follow up status showing the above detailed changes.**

Dr.Sara Mahmoud, MD

Dr.Sara Ali

CT Scan
[DOCTYPE:CT_SCAN]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Eman Taha Hassan Ali	Date : 19/11/2024
Exam : C.T CHEST	Age : 67 Year(s)
Medical ID: 5331/2019	Pat. ID: 39719

FOLLOWUP NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS: -

Known case of cancer right breast

Compared to last CT study dated 1/8/2024.

Chest Findings:

- Status post right breast conservative surgery with mild overlying skin thickening and underlying heterogeneous density
- **Still noted** the middle and lingular lobes fibrotic bands with no significant changes since last study likely post therapeutic sequelae
- **Stationary course** of the previously seen small pulmonary nodules with the largest seen at apical segment of right lower lung lobe paravertebral in location measures about 1.1 cm.
- **No time interval changes of the previously noted** bilateral diffuse emphysematous changes with small foci of air trapping.
- **No newly developed lesions**
- Subcentimetric enlarged precarinal lymph nodes are seen
- No evidence of pleural or pericardial effusion
- Normal non-enhanced CT appearance of the heart and major thoracic vessels.
- Small hiatus hernia is still seen
- Bilateral elastofibroma dorsi are still seen.

Abdomen & pelvis:

- The liver is mildly enlarged in size showing diffuse hypodense low CT parenchymal attenuation and regular outline. No focal lesions along the basis of the non-enhanced study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Gall bladder bed shows metallic surgical clips of previous cholecystectomy.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- **Stationary course** regarding right suprarenal nodule measuring about 17x14 mm.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Bilateral renal cortical cysts are noted largest is at left kidney measuring about 3.5 x 3.5 cm.
- Normal non enhanced CT appearance of the pancreas, left adrenal gland, aorta and I.V.C

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Eman Taha Hassan Ali	Date : 19/11/2024
Exam : C.T CHEST	Age : 67 Year(s)
Medical ID: 5331/2019	Pat. ID: 39719

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Skeletal examination revealed:
  - " Dorsolumbar spondylotic changes.
  - " No sizable lytic or sclerotic bone lesions.

OPINION:

Overall stationary course since last study

Doha Ahmed

Dr. Ibrahim Abo El Magd, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Iman Taha Hassan Ali
Branch : Dokki
Ref.Physician Prof : Gostaf Rosch Hospital

Date : 05-11-2025 11:58
Medical No. : 242098
Ins.Company : تعاقد مستشفيات

$Tc^{99}m$ MDPSKELETAL SCINTIGRAPHY

History

- A 68-year-old patient, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Marked degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

NILE DIAGNOSTIC CENTER

جمهورية مصر العربية

بطاقة تحقيق الشخصية



ايمان

طه حسن على

٤١ ح الباسفجى-ش الترعة البولاقية

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DMS ٤٠٧٩

Patient's Name:	**Iman Taha Hassan Ali**
Date:	**2026-09-05**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

Known case of breast CA. compared to the previous study dated 5-5-2026, the current study showed:

FINDINGS:

(I) Chest:

- Still noted right breast soft tissue density inseparable from underlying pectoralis muscle measures 2.7 x 2.9 cm for sono-mammography.
- No appreciable time interval changes regarding the previously seen right lower lobe posterior segment pleural based pulmonary nodules 12 x 10 mm .
- No sizable hilar or mediastinal lymph nodes .
- No sizable pulmonary masses or consolidation patches
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Still seen bilateral elastofibroma dorsi.
- Small hiatus hernia.

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous low CT density no focal lesion.
- Still noted cholecystectomy clips.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys (apart from bilateral cortical cysts), aorta and IVC.
- Normal CT appearance of the spleen .
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.

OPINION:.

Stationary course since last study

Dr.sherif Wagdy